

The Social Radars

S1: Edith Elliot, Co-Founder & CEO of Noora Health

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Jessica: I am Jessica Livingston, and Carolynn Levy and I are the Social Radars. In this podcast, we talked to some of the most successful founders in Silicon Valley about how they did it. Carolynn and I have been working together to help thousands of startups at Y Combinator for almost 20 years. Come be a fly on the wall as we talk to founders and learn their true stories. Today we're talking with Edith Elliott of Noora Health, a nonprofit startup, which went through YC in 2014. The nonprofit world needs startups just as much as the for-profit world, if not more so. And Noora Health is one of the most remarkably effective of this new generation of nonprofits. I think you'll learn a lot from Edith's story. Enjoy. So Carolyn, we are here today with a very exciting guest. We have Edith Elliot of Noora Health, which is a nonprofit that Y Combinator funded in the winter of 2014. Hey Edith.

Edith: Hi.

Jessica: How are you?

Edith: It's so exciting to see you both. I'm great, thank you. How are you?

Jessica: Good. I'm so excited to talk to you today and have you on the podcast because you're our first nonprofit. And so I'm interested in not only hearing your story, but also learning a little bit about the difference between nonprofit doing exciting things and a regular startup, which of course, Carolynn and I for-profit startups, Carolynn and I are used to working with mostly. So, first I want to start out in a different way today. I just want to first quickly ask you, because I'm going to describe it badly, but I'll describe it. Noora Health helps train families and caregivers, mostly in India at this point, on post-operative, caring at home and caring for your babies after you've had a baby. Can you describe it better than me? Just quickly.

Edith: No, that was excellent. Yeah, so we focus on training and upskilling family members in the care of their loved ones, and we do this across multiple different condition areas. The largest body of work has been in the maternal and newborn care space, so both antenatal care, which means when you're pregnant and before you've had your baby, and then postnatal. So helping to prepare not just new mothers, but really seeing the mom as a patient as well, and making sure that fathers, mother-in-laws, grandmothers, the ecosystem that is around the mom and the baby at the point of the baby being delivered, are skilled and trained and understand what both the mom and the baby are going to need in those critical 30, 60, 90 days and up to really up to a year post the baby being delivered.

And we also work in cardiology and cardiac surgery and general surgery and all sorts of different areas, but the core concept is family is there, they're able, they're capable,

they're willing, and are largely untapped. And you have this brilliant resource that is in these health systems that are otherwise really under-resourced and strained that is able, capable and willing and eager to support,

Jessica: Eager being the operative word, because it's their family.

Edith: Exactly. It's their loved ones, and no one knows and can take care of a patient honestly better than their loved one. And honestly, the same is true in a Western hospital as well. If you think back to maybe when you've received care, a loved one has received care, it can be a really scary thing. And discharge instructions are horrible. And you think of a thousand questions that you wish you had asked the minute you leave the doctor's office or the minute you leave the surgery room or whatever it might be, or the hospital after a surgery. So we see this as a universal solution. And across condition areas, the theme is reducing complications, reducing readmissions, reducing mortality, and improving health seeking behavior. So making sure that if someone needs to go to the hospital, they know when that is and what to do in the event of an emergency or in the event of a complication popping up. And behavior change is a big component of it as well.

Jessica: Now I want to talk more in depth about what Noora Health does, but I want to interrupt because we're at the top of this podcast, and I want the listeners to have this in the back of their mind because I think that Carolyn, you might not have this in the back of your mind, Noora Health saves lives and you can assign a dollar value, can you not, Edith, to that life, how much roughly does it cost to save a life?

Edith: So right now in 2022, our estimated cost per life saved was just over a thousand dollars. And in the grand scheme of things, that's actually quite low in the global health space. And over time, as we scale and as the program is able to reach more people, that cost per life saved goes down fairly dramatically. And our estimate over the next say five years is that that will go down to just over a hundred dollars per life saved.

Jessica: Wow.

Edith: Which would be wonderful. If we can hit that target, it would be exciting.

Jessica: A hundred bucks. I know you're not there yet, but you're at a thousand bucks per life, roughly. Even that is much lower than the norm, is it not?

Edith: It is. What gets really tricky and complicated, and if there are any global health experts who are listeners, how to measure that is incredibly complicated. It's not a simple equation that you can... what we do is we take our research that shows the reduction in infant mortality and maternal mortality and use that data. What we do at Noora is just take our fully loaded budget and divide by the number of people that we've reached, knowing what our reduction in mortality is. But there are far more complex ways of doing this. And so you really have to dig in and you have to understand when someone gives this type of statistic, what are the inputs that they're putting into measure that some of the lowest, current lowest estimates for cost per life saved usually hover around

like \$1,500, \$1,200. But again, you have to dig in and see what is going into... how are people calculating it?

Jessica: I didn't mean to put you on the spot, but I just wanted people, as they're listening to this interview and we're learning more about Noora Health and Edith personally, I wanted people to remember what she is doing because it really is, yeah, a unique thing. That's just the backdrop.

Edith: Thank you.

Jessica: So take me back because we met you in winter 2014, but take me back to how you got started. What was your background? How did you get into this?

Edith: We didn't intend to start an organization and certainly not a nonprofit. That was not my dream. That was not what I thought I would be doing. Noora started in sort of a different way. Myself and my co-founders met while we were grad students at Stanford. My co-founder and co-CEO, Shahed, was in medical school. I was getting a degree in international policy and pre-Noora and pre-grad school, I had worked for a number of nonprofits and had left and gone on to get this degree because I was sort of jaded by this space and had been left curious why things felt very dysfunctional. And the organizations that I worked with were highly functional within the space. These were A plus nonprofits, massive organizations that do really remarkable work. But the disconnect between the donors, and in this case, most of the donors were very large aid organizations, multilaterals, bilaterals. It's like a complex ecosystem.

The disconnect between what they thought was needed, in reality, on the ground, and in the communities where the funds were eventually flowing and the work was eventually being executed, there was a very significant disconnect between those two. And I thought I would go and work on the donor side. I wanted to see what was broken on the donor side so I could have more of a holistic view. And had gone to grad school with a pretty open mind and not sure... I just knew that I had kind of reached a... I don't know, I reached a point in my career where I needed more and wanted to take a step back. Shahed and I met, and our two other co-founders, Katie and Jessie, we met at something called the design school at Stanford, which teaches students about human-centered design. And in our case, the course was called Design for Extreme Affordability.

And the idea was that you used design thinking to solve a problem for an organization or a government or an entity that needed an extremely low cost solution to a big problem. And so in our case, we were introduced to a doctor in India who opened the doors to his hospital and his health system. And the problem that he posed was overcrowded hospitals, family members in the way and crowding the hallways, crowding the waiting rooms, asking a lot of questions, eating up this precious doctor and nurse time. In India, patients, you're lucky in a government hospital if you get two minutes with a doctor or nurse during your hospital stay.

Jessica: Oh my gosh.

Edith: There's these very rushed interactions. And so what you would have is, let's take cardiac surgery. Someone comes in, they have a successful surgery, but then they're going home and the health system knows that they're traveling hundreds of kilometers away from the hospital and things can really fall apart. You can have a complication, the wound can get infected. You don't do your physical therapy exercises, you don't take your medicines correctly because you, as the patient and the family member, were handed a bag of pills and a piece of paper that you may or may not be able to read depending on whether or not that piece of paper was in your language, whether or not you can read, there are so many factors that could go into you not being able to then provide proper aftercare to yourself or to your family member.

And we looked at all of this and we used design thinking to interview hundreds of patients, families, doctors, nurses, everyone from the staff who cleaned the wards, to the CEOs of these hospitals or the government bureaucrats. And what we found was, as I said at the beginning, was this unbelievable untapped, compassionate resource who was overlooked within the system and when harnessed and engaged and given skills training, not just given information, not patient education, but actually up-skilled in a really intentional way, in the right way, at the right time, in the right language, and with locally contextualized imagery and with the medical information demystified that we could drive really exciting outcomes.

And so we did this as a class project. And at the end, this doctor was really excited with the outcomes. The hospital had lines around the wards, family members, really eager, they wanted to take it to the rest of their hospital chain. And this was back in 2012, so a little bit near the end of 2012 before we went through Y Combinator, we were still in grad school. So this was just eating up all of our time and it became our obsession. And fast-forward a little bit, we did some research, we had some initial findings that were really exciting. We saw a 71% reduction in post-surgical complications for cardiac surgery patients, which was huge.

Jessica: Wow.

Edith: Yeah. And we thought, wait a second, this is a solo cost because it's using, it's a train the trainer approach. We train existing healthcare professionals, so doctors, nurses, typically nurses who are already employed by the health system. It's seemingly simple, complex in execution, tweaked to the system that really any health system can do. Certainly if these under-resourced hospitals in India can do it, then other hospitals can do it. And we thought this is low cost. I thought back to my previous life in the nonprofit space, and it was so low cost, seemingly really scalable and highly impactful. We felt like it was almost a moral imperative to carry it forward.

So I graduated and took a fellowship at Stanford so I could have some kind of income and stick around and still be working on this. Jesse and Shahed, my now co-CEO, lived in India for the summer and took a bit of time off from med school and near the end of 2013, it was pretty clear that we had other hospitals coming to us wanting to implement this. We looked around to see were there any other organizations who were doing this that we could give this to, that we could say, okay, hey, we did this cool thing. Here are these resources if you want them and we couldn't find anyone.

Jessica: You couldn't find any consulting companies that wanted to do this?

Edith: Look, patient education and this is typically seen as just a tiny sliver of a more comprehensive package within healthcare systems. And it's hard to prioritize it because it's not super revenue generating or it's not seen as one of the core... typically you're more focused on how to, in the case of a for-profit hospital, how to drive revenue or how to improve patient satisfaction, caregiver satisfaction was definitely not as much on the radar, but we saw we were reducing complications, we were saving the system money, but we knew it was going to take time to prove out the value. And so that was when Katie, our co-founder who now is not with us full-time, but who we're still very close to, she heard through her friend that YC was... put out a call for nonprofits and we just assumed we'd have, at this point, we weren't even registered as anything. We didn't have an entity.

I remember the first little bit of money we got from Stanford, it was coming to us because we were still affiliated with the university. And I'm like, what's Y Combinator? What is this? Okay, but sounds really cool. They'll let us focus for three months. As I looked into this, I thought, well, this would be kind of a cool way because of my disillusionment with the sector, I thought this would be a really cool way, we don't even know if we want to start an organization. We don't know if we want to start a company. We don't even know if what we have has enough legs to start a company. And we also simultaneously had some US hospitals after hearing that we were reducing complications, readmissions, things that were so expensive to their health systems, they wanted to work with us.

And so we thought it would be an exciting way to focus. So Jesse and Shahed took a leave of absence from med school, which is a big deal, a three-month leave so they could focus. And Katie and I decided to quit our... I stepped away from the fellowship. She was doing some research at Stanford. I remember that conversation with... well, the interview blew my mind because I had no idea what to expect going into it. I was so naive about Y Combinator and what we would be experiencing in the interview, definitely. My head spun around. I had no idea because most of my experience with any sort of funding had been so different, long-drawn...

Jessica: Longer than 10 minutes?

Edith: Just being [inaudible 00:15:48] with questions. And it completely changed our life. It completely changed our life.

Jessica: I am going to quickly just for the listeners mention, because most people know that we fund startups, but in 2012, Paul decided, hey, it could be really cool to do an experiment, which we were always doing, and see if we could run an appropriate type of nonprofit driven by software and that kind of thing through YC. Could we be helpful? Because he believed that nonprofits faced many of the same challenges in running a company or technical challenges that a for-profit would have and maybe it would be a good idea to bring them into the community and run them through the three-month program, so we did.

When Watsi was our first nonprofit and they did crowdfunding for life-saving operations, I think that they were mostly in India too, and it really saved lives. So we thought, this is a great success, let's do some more of these. And then we started proactively calling for nonprofits to apply back then. And I'm so glad you say it changed your life because I feel like if you hadn't done that, maybe you would've gone on to doing something different and you wouldn't be saving so many lives. So tell us about... I want to hear about your experience at YC. I want more details.

Edith: Yeah. Well, again, I didn't have a tech background. I didn't come from the space. Yeah, I lived in the Bay Area for a while, but I was studying policy at Stanford, totally different. This is a different world. And had heard... I had the preconceived notion of what to expect. A bunch of dudes in hoodies over computers hunched over and what was the show? Silicon Valley the show was on around that time. And sure, some of the stereotypes hold true, but so many didn't. What we walked into was this incredibly excited, energetic, amazing community of people, of doers, of people who are changing the world, who want to create and will create massive change in each of our lives in some way, shape or form, and who had the most incredible willingness to welcome an outsider like me into this community and be supportive and helpful in making our dream a reality and accelerating our dream through the use of technology in ways that I wouldn't have been able to conceptualize.

And I would not have been able to see into the world and ask us critical questions that certainly we would not have been asked had we gone through more typical incubators, say for social ventures, and we have been through, post YC, we went through a handful of fellowship programs that were fundamental to our now success. And we had unbelievable experiences there as well, but very different. When I think about YC, it laid the groundwork. So many of the values that we hold as an organization and as a company, and we do see ourselves as a company, not a nonprofit, are based on what we learned during YC and how we were pushed during YC. And the questions that each of the partners would ask, the walks with Paul, where he would try to understand and dig in on the complexities, but then help us zoom out and find simplicity when typically in the nonprofit space, donors will drill into the complexity. That for us, was really critically important at the time. And just focusing, focusing, focusing on a goal.

Jessica: Well, a lot of people say focus is one of the most important things that happens at YC.

Edith: And knowing what to focus on.

Jessica: Yeah, choosing the right thing to focus on, which we, I hope helped you do that. Paul helped you do that.

Edith: Yes.

Jessica: I do want to, if it's okay, I want to talk a little bit about donors because they're such a different animal than investors and there's so many things that I only would've known from being exposed to all these startups through YC. At the end of the three-month program, of course we have demo day where startups in the batch present to hundreds

of investors and start the fundraising process from there. Now, one of the naive things that we didn't... duh, think about when we started this experiment in 2012 is like we kind of said, oh, there's going to be tons of rich investors in the audience, you can just raise money from them.

Which was so naive in hindsight. Yes, there are probably some investors that really felt a connection with a particular nonprofit and maybe donated, but investors are at demo day for one reason, to make investments in for-profit companies for their portfolio. They're not there for personal reasons. And I think unfortunately, sometimes the nonprofits sort of come away from that day feeling like, I didn't get many donations, people were too busy to talk to me. What was your experience like on demo day? And then how about how you got your first donations?

Edith: Yeah. Oh, thinking back to demo day, my heart does a little bit of a pound. No, in a good way.

Jessica: Oh no.

Edith: We learned how to tell our story and how to pitch at Y Combinator. And that was such an invaluable, because so much of fundraising for nonprofits is how to sit down and succinctly be able to win the person on the other side. One, know when and if the person on the other side of the table wants to hear your pitch. And two, be ready with something that is clear, well articulated and doesn't overwhelm. And again, I think in this sector too often it becomes too complicated when really the core essence of whatever it is that you're doing is really what people want to hear about first and then can dig in and ask questions. And so Demo Day for us was a remarkable lesson, like a masterclass, both in the prep of it and the day itself, in telling the story effectively.

And I remember we were practicing, and Paul knew our pitch down... Jessica, I remember you and Paul were sitting right there, and I said one word off a little bit, I don't know. I said was instead of... who knows what it was, and he knew down to the word, and I've always been... and Jessica, I remember you saying, I think we should have her talk about this a little bit more. And just the way that you so thoughtfully and elegantly helped us put our story together, whether or not people in the room were ready to hear it, has served us well. Even just this past week, I had to tell our story many, many times at this conference that I was at. And it's the same, that line, a willing, untapped resource, that all came from Demo Day. I can still recite our pitch.

Jessica: Oh, that's awesome.

Edith: Yeah, it all came from...

Jessica: 10 years old.

Carolynn: That is awesome. [inaudible 00:23:24] 10 years old.

Edith: 10 years old.

Carolynn: That's a 10-year-old line.

Jessica: Paul can hear the clank of a one-off word like no one else I know.

Edith: That's remarkable.

Jessica: That doesn't sound right. It is remarkable.

Edith: We raised money.

Jessica: We did raise money from Demo Day.

Edith: Yeah, we did. We didn't have any handshake deals. We didn't have anyone say, okay, I'm going to give you X on that day but we met two donors who we had follow-up conversations with and who subsequently became, and still one continues to be one of our largest funders to date, well, two actually, if I count one other group, our largest, and then one of our second largest to date. The one I'm thinking of is an anonymous funder so I can't say who, but they truly, right after Demo Day, we didn't have... they were in the audience and then someone from their team came up and said, we want to learn more and then that door was open and it was a remarkable relationship that started and continues to this day.

Jessica: Well, that makes me happy because here I was being embarrassed for us not realizing that there wouldn't be proper donors. So that makes me happy to hear that. I do want to talk about donors though, because this is sort of a revelation that I didn't understand. A lot of these donors, let's just say they're investing a million boxers, a large amount of money, they can put it as like, we only want you to use this money for X and that's called a restricted donation. And that hamstring the nonprofits who are running their nonprofit like a business and they know best where they need to spend the money, but the donor has these preconceived notions about what's most effective, and they place these super restrictions on the donations, right, Edith?

Edith: Yeah. Yes. It's terrible.

Carolynn: Can I ask a follow-up question really quick? Are they allowed to restrict your use of it on operations?

Edith: That's the main point.

Carolynn: Okay. So in other words, you have to pay your bills, you have to pay your rent, you have to pay your employees. But if a donor gives, the million dollar donor example here says, "You can't use any of my money for operations." And you're like, well, I guess I can only use it for this thing-

Edith: You can only use the program.

Carolynn: ... but I can't can pay my rent and I can't pay my-

Edith: And this is one of the massive parts of the system that we see as being extremely broken so that was a perfect description and a perfect follow-up question. And many organizations who only have restricted funding and who rely on restricted funding have a problem where they'll run into issues where one, they certainly don't have any funding they can use for innovation or big bets or doing something they said that they were going to work in this set of five hospitals, and they realize halfway through the grant period, let's say it's a three-year grant, that actually in this hospital, there's a bureaucratic issue and it's going to shut down. And in this hospital, X, Y, and Z, sometimes you have to actually give the money back. You can't say, oh, we found this other location where we can... Oh yeah, yeah.

You hear horror stories, you hear horror stories. Donors don't like to pay for rent. Donors don't like to pay for 401k plans. Donors don't like to pay for... the average donor, I mean, right? So we pretty early on, and this was in part because of my past experience and my pre-Noora life, we said we're not going to take restricted funding. We're just not going to, and that's going to mean that we probably have to say no to money, which is not easy, but we're going to see if we can make this work with unrestricted funding. And we were able to do that because we met a group of remarkable philanthropists and foundations who, of which there are very few... more and more, because this group in particular, among others, have started to really beat the drum of, hey, unrestricted funding is the way to go. Unrestricted funding is the way to go.

And we tried to raise money again, this was something that we learned through YC. We didn't do it like Chase did at Watsi where he would do these rounds, right? Similar to a for-profit, we didn't do it like that. We had more of a rolling fundraising strategy, but one, we knew we didn't want to spend all of our time only fundraising because we had stuff, we were a small team, scrappy-

Jessica: [inaudible 00:27:49] actual work to do.

Edith: [inaudible 00:27:50] work done. Exactly. And so we were able to find a few funders who invested based on metrics and milestones, who invest based on a conversation, not some long-drawn-out written grant that we're... a donor, typically how it works, Carolynn is a donor will say, we're going to put out what's called an RFP, a request for proposal for work in the maternal and newborn care space. Nonprofits then apply for that with a very typically long grant. Then there's some kind of back and forth and you're lucky if you get the money, if you're the nonprofit that is selected. By the time you found out if you've got that million dollar grant, you've invested a lot of time, a lot of your organization's time. We are terrible at grant writing at Noora. We're horrible at it. Our fundraising team is me, my co-founder, and one and a half now people. That's it.

Carolynn: How many total?

Edith: Our total headcount is just over 400 now. And we're growing pretty quickly this year. But not even really... I call it two, two people and no grant writing, they're focused on... sometimes we will respond to, if it seems like a donor who's aligned and it's unrestricted and it's a short application, then of course we'll do that. But we focus, our fundraising approach is more relationship based. We focus on getting to know a foundation. We do

much better when it's a conversation. And then, yes, there's a diligence process where they, similar to in the for-profit space where you're going to be asked for a ton of documentation, you're going to be asked for a bunch of... which is good, yes, please come.

Oh, we're very open...come pick away, help us figure out, look under the hood, whatever needs to be done. And then an unrestricted grant is made based on milestones, based on our objectives. And we use the same milestone document for all of our donors, the majority of our donors. If somebody has a template, then we just copy and paste and pull it in. And then our commitment back to the donor is that we will, on a quarterly basis, report back on how we've performed against those metrics and milestones and send them whatever. If they need a little more information than that, fair but we will not do a long grant. A lot of times you have to write a book basically about what you did and we don't do that.

Carolynn: That's a full-time job for a lot people, grant writing is a real career-

Edith: Yeah, totally.

Jessica: So it's my understanding, and Edith, correct me if I'm wrong, but the biggest complaint of so many nonprofits is that they just spend all their time for the whole year fundraising, it's a never ending thing. And I remember Chase of Watsi used to come and give advice to the new nonprofits that were in the programs of treat your fundraising as a for-profit company would do their fundraising rounds. We're going to go work for a year and then we're going to set aside this month to fundraise and I remember that was good advice because it sucks up all your time, but I'm also going to go out on a limb, Edith, and I'm saying this, not you, but I feel like there's no competition among donors.

At least investors, Carolynn, have some competition to get in deals and they have to behave and they have to sure be fair and all of that. And sometimes I think that donors just want to micromanage and get into all the minutiae and ask questions that a regular investor would never care about asking, like how much are you spending on this? And how much are you spending on that? And that's so time-consuming.

Edith: 95% of our funding is unrestricted. We have taken, during COVID we took a pretty good amount actually, of restricted funding, that was for our COVID specific programming. But we did that because one, there were a lot of donors trying to move money quickly for this specific problem. We had a solution that we saw was working and we were willing to say, okay, well we know we're going to spend the money within this period of time on this but it was loosely restricted. And I will also say that, and again, I want to tread carefully, the few times that we have taken restricted funding, and we actually recently had an instance of this where we were... it's called a sub-grantee. So a large funding institution, whether it be [inaudible 00:32:19] or USAID or Gates Foundation or one of these big entities, will make a large grant to one of the large nonprofits.

There are these huge, large implementing nonprofits around the world and then they sub-grant to smaller organizations on a project. And so we've, in a few instances, have

taken the sub granting money and it's really hard. Our team doesn't know... we're not a good partner in that always because you're constrained in ways that you're not used to being constrained. And our team, which we've built on values of move fast, make things people want, a lot of our core operating principles are based on values that just misalign at times with the more traditional funding streams that come through. But we've also been really lucky to find ourselves in community with and in partnership with donors who are taking big bets and it's time restricted, which I don't count as restricted.

One, a big prize that is a consortium of donors who come together and make big bets, it's through TED. It's called the Audacious Prize. And Chris Anderson and the Audacious Team are just remarkable and have made it really easy for these 10 donors to come together and make a significant capital infusion into an organization with the idea of it being like you're going public moment or your moment to have a huge leap in what you're able to do. And so in our case, it was 50 million over six years to reach 70 million caregivers, was the investment. It's intended to help us scale the work in India and Bangladesh where we were already working when we won this, and then expand to two additional countries. So Indonesia, we've just hired our first few people and our starting expansion in Indonesia, and originally the plan was Nepal, but we're actually exploring different opportunities and most likely we'll still work in Nepal, but potentially one additional place.

And this is what I mean by... so it's time restricted. We have to spend it within those six years, but we can go back to this remarkable group of people and say, hey, this is working, this isn't working. Here's what we're doing instead, we just got this big opportunity with the government to fill in the blank, we're going to sprint down that path, or we're actually seeing this program doesn't work, so we're not going to do that anymore. Because it's trust-based funding, we won't be in a position where we have to give the money back, like I said, sometimes it happens, it's a conversation, which is really refreshing.

Jessica: I want to ask two questions. The first is, what advice do you have for anyone considering starting a nonprofit? What general? What would you be sure to tell them? And I also want to ask you what advice you have for donors?

Edith: Advice for anyone wanting to start a nonprofit. First look at the ecosystem. Ask yourself, this is the case for a for-profit as well, are you solving a massive problem that will make a difference in many people's lives that you are passionate about? Because it's a long road. This is not easy. I feel so blessed to be able to call this my life's work and that we've had the opportunity to have the kind of support behind us that we have, that allows us to see this as our life's work. But you should first take a look around the sector and see, is anyone else doing something similar? Because there is so much... Jessica, you talked about the competition among investors and how that in part fuels the urgency and Carolyn, the time-bound nature and everything, the competition within and the money relationship is with the nonprofits.

So you're competing for that grant money and yes, for-profits are competing for investment, absolutely but the stakes are very different. And so if you can find yourself being an innovator from within, in an organization that already has a footprint of some

kind, then that's an awesome path to take. But if not, then go for it and start a nonprofit. There are so many problems, massive problems in this world that traditional markets can't and will not solve. And there are skeptics about nonprofits, absolutely. But when we looked at... we were working with some hospitals in the US and at first thought, oh, we'll make money over here and then reinvest it in our programs in India, Bangladesh, and other parts of the world and we see this as a global solution. And our hope is that very soon we'll be able to work with Western markets as well or with more financially funded systems.

But when we looked at every dollar that we were able to raise, we could take so much farther in India and be so much more innovative and creative with how we were testing out the solution compared to the US. What would cost a dollar in India, in terms of us being able to drive impact, was exponentially more in the US and that's just in the case of healthcare. So I tell people, go for it and get creative and don't become too jaded because there are ways... there are brilliantly thoughtful and remarkably generous people, but understand what your values are and what you are and you are not going to be willing to do. I think there's a path that you're told you're supposed to take and you don't have to and it might not work. And like I said, I'm sure we left money on the table, a lot of money on the table, a lot of opportunity on the table, but it's what's worked for us.

And my recommendation to donors would be to give unrestricted, enter into trust-based relationships with your grantees. Get to know them, get to know the work. I'm not saying just write a check and call it a day. What I'm saying is trust. And if you don't trust the organization and if you don't trust that the organization is the expert in the room, then you might not be talking to the right group or you might need to take a look at yourself and really ask yourself, are you the expert? And do you want to be the expert in this? If not, go find the experts and trust them to go do the work. And don't ask them to write long reports.

Carolynn: Yeah, that actually seems like... They must be employing people on the other side who have to read all that, right?

Edith: Absolutely.

Carolynn: Yeah, right? They cut all that out and then everybody's happier.

Edith: There's some amazing... we've made it public knowledge that MacKenzie Scott was part of our audacious group of donors and the way she's doing philanthropy is so cool.

Jessica: Did you get to meet her?

Edith: We haven't met. No, we haven't met, Nope, we met-

Jessica: Even more intriguing.

Edith: We met the folks who do the diligence for them.

Jessica: She just seems so cool. She's writing checks all over the place to places-

Edith: It's amazing.

Jessica: Yeah, that's really trust-based.

Edith: It's amazing. And if you're a funder, if you're someone listening to this who has money, look at... there's a group called the Big Bang Philanthropy Group. It's a group of foundations who really have come together and have a core set of values and an ethos around unrestricted giving. Look at what they're doing, look at what MacKenzie Scott is doing. Look at what some of these really innovative philanthropists are doing. Look at what Paul and Jessica do in their giving. It's remarkable what this trust-based relationship can unlock, I think, in terms of innovation in the space.

Carolynn: Okay. So my question's kind of pedestrian compared to what we've just been talking about, but I'm always very interested in how people come up with their name. So can you tell us about Noora?

Edith: Yeah, absolutely. At first it was called *Care*, and actually it's still called... in the country, you'll see the Care Companion Program, not Noora Health very often. But we had a brainstorming session with Paul and one of the names that we came up with was Care Squared, I think back. And I'm like, oh geez, thank goodness that didn't win out. But we looked back at our interview notes from meeting different patients and families along the way and had this long list of people's names. And then one of the names that really stood out was Noora. Noora means Light in Urdu and was the name of one of the first women who we met and her story was very emblematic of the stories that we heard over and over and over again. It was a tragic story and we thought it was nice... both the meaning of the word and a nice sort of tribute to the work and also to the prospect of something better.

But I remember we went to Paul with the name and he said, "Well, I don't don't know how to spell it. And I usually don't like names that I don't know how to spell. I don't know what it means. And I usually don't like names that I don't know what they mean and it's long, but I love it. It's perfect. Noora Health.

Jessica: Wait, Noora?

Edith: Noora Health.

Jessica: He said Noora was long?

Edith: The long... oh no, sorry. And I don't know how to pronounce it. I don't know how to pronounce it, which is never a good start because you want people to be able to remember it and pronounce it. And he was like, but I love it.

Jessica: Speaking of names and recognizing names, let's pretend I'm someone who thinks, okay, I want to do a donation, I want to help people. Where do you start? Say I only have 200

bucks or 400 bucks. How can you judge? I feel like people just donate to the name brands that they recognize, the United Way and all of that. And I feel like there's got to be something that helps you judge, like efficient, impactful nonprofits. Do you have any ideas on that?

Edith: Yeah, it's a great question. Absolutely. One, I always tell people, find something that you can feel connected to, that your heart feels connected to, that when you hear about Ignites your excitement. And then I always encourage folks to look at whatever the organization has made public. Of course, it's always best if you know somebody within an organization or who's worked at the organization, that's a fantastic way to find out what the reality is in terms of... organization will say, for every dollar we raise, we do X, because it's an easy thing for philanthropists or donors to grab onto. But really peeling apart, this is why sometimes we get uncomfortable saying our cost per life saved because you can't provide in that one sentence all of the context for how that number was calculated. And we tend to be too scientific at times and too wonky in wanting to get into the details.

If you're donating and you've been told that that dollar is going to X, try to dig in a little bit. If you're curious, otherwise, trust. Otherwise, just trust, okay, this is what they're saying. And if I feel my passion and I feel confident in this, yes, do your diligence. But if you're talking about a couple of hundred bucks, then you can afford to make that bet essentially. Ask yourself, can I afford to make this bet? I love talking to people who are interested, sure, in Noora, but who are interested in any other organization helping make those connections. I feel like it's part of our responsibility as nonprofit leaders to say, oh, Jessica, you're not interested in Noora, but it sounds like you might be interested in X, Y, and Z. Do you want to meet those folks? I'd love to introduce you to them.

Jessica: That's a great way to end. All right. We better let you get back to your important life's work. And to the listeners out there, if you do consider lives valuable and you want to make the world a better place, check out Noora Health and make a donation. All right, Edith.

Edith: Thank you so much.

Jessica: Good luck and keep us posted. Thank you-

Edith: We will.

Jessica: ... for coming today. Thank you, Edith.

Edith: Bye.

Jessica: Carolyn, wasn't that so fabulous to talk to Edith?

Carolynn: It was great. She's so articulate and obviously so experienced. I feel like she could have said a lot more about donors and fundraising, but that was really, really good.

Jessica: I'm telling you, Carolynn, there's this world that most people don't know about that is just very fascinating and I feel like there's some change that's necessary and I'm glad that Edith has things figured out and is like, we don't take unrestricted money and that's that. We didn't even get into, I wanted to ask her what it was like when she was in India and what it was like going into one of these hospitals and training people? And I didn't get to do that, Carolynn. I feel like there's just too much to talk about in an hour. But she's just amazing. She's such a role model for so many people, and I just really feel like there should be more nonprofits in the world and if you can do it, you should do it.

Carolynn: Yeah, her superpower is probably that she comes across as... she mentioned the word trust 15 times, and her whole vibe is competence and trust, I can totally see how she's so successful [inaudible 00:46:22].

Jessica: And calm too.

Carolynn: Trust her.

Jessica: I love that quality in someone, I don't have it. But that was a lot of fun and a little bit different than our usual startup conversations. I feel like I learned some interesting things in that one. So that was a lot of fun. All right, well I will see you next week.

Carolynn: Sounds good. Bye.